National Cancer Control Programme (NCCP) 00696 · clinician draft

Paclitaxel + cetuximab every 4 weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1 and 15 · cetuximab followed by paclitaxel

Active medicines: Cetuximab + Paclitaxel

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	60 minutes before cetuximab NCCP Table 6; this antihistamine also covers paclitaxel on combined-treatment days. Do not duplicate it.
Dexamethasone	8 mg IV	60 minutes before cetuximab NCCP Table 6; this steroid also covers paclitaxel on Days 1/15. Reconcile total steroid dose and diabetes/sleep advice.
Famotidine	20 mg IV	30 minutes before paclitaxel NCCP Table 7. Consultant may omit after review; the source notes uncertainty about hypersensitivity risk when omitted.
Low-risk / rescue antiemetic	Individual prescription	If required after accounting for infusion steroid No routine moderate-risk course. Persistent symptoms warrant review.

Additional phase-specific support

- Give an antihistamine and a corticosteroid at least one hour before cetuximab; continue this premedication at subsequent doses. Reconcile steroids with any antiemetic prescription to avoid duplication.
- Ask specifically about earlier antibody reactions, red-meat allergy, tick bites and any cetuximab IgE result. Premedication does not eliminate severe or delayed reactions.
- Use a local EGFR-inhibitor skin plan, moisturiser and sun protection; assess painful, infected or intolerable rash and nail changes promptly.
- NCCP allows observation to cease after two reaction-free cetuximab doses, whereas current Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Agree and document the governed observation plan; do not automatically remove observation.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- Consider prescribed analgesia for paclitaxel-related muscle/joint pain. Assess neuropathy before it becomes disabling; do not wait for a routine cycle boundary.
- Both medicines are low emetic risk in NCCP v2; do not carry forward an unsupported moderate-risk antiemetic schedule. Day 1/15 cetuximab steroid/antihistamine premedication also covers paclitaxel; Day 8 needs its distinct plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00696 v2 · pNCCP 00696: treatment and supportive-care sections; NCIS v8 pp4–5 where used · NCCP 00696 and current NCIS/local supportive-care guidance

Sources checked 2026-11-10 · SACTCheck v0.84.47 · Regimen-level drafts; prescriber and oncology-pharmacy verification required